Audience Q&A and Safety Answers

Short, defensible responses tied to actual evidence rather than assumptions.

Does the AI make medical decisions?

No. It extracts researcher-defined study fields from synthetic descriptions. It is not designed to diagnose, prescribe or recommend treatment. Human confirmation is required for Excel export.

How accurate is it?

We have one documented synthetic Phase 4 case with a 4.6/5 independent AI quality rating. That is not an independently validated extraction accuracy figure. Two human reviewers and a larger synthetic benchmark are still pending.

Is patient data safe?

We prohibit real patient data in the demonstration. Current regex screening is best-effort only; Telegram sees incoming data and voice is sent to Groq before words are screened. This prototype is not suitable for real PHI.

Why should researchers use it instead of Excel?

The intended benefit is easier capture, clearer exception handling, and an approved XLSX that researchers can use. Whether it saves time must be measured in a structured pilot.

What happens when voice transcription is wrong?

The researcher confirms transcript text before extraction, and schema/evidence checks can flag mismatches. In the observed demo HbA1c was misrecognized; the system alerted, the researcher corrected the value, then approved export.

Why two Gemini calls?

The first call extracts the structured values; the second applies an independent QA rubric. A model score is still not a ground-truth adjudication and needs human calibration.

What does LangSmith currently trace?

It has a verified metadata-minimized QA checkpoint with judge scores and alert metadata. It does not yet log every individual stage or all Groq and Gemini calls.

How much money will this save?

We have no measured savings or ROI. An eight-minute manual record is a planning assumption; the team will time paired synthetic record entry before claiming a benefit.

Who would buy this?

A research unit or principal investigator is the working buyer hypothesis. Customer discovery is part of the requested pilot.

Why one workflow if the written guide says five?

The instructor explicitly approved one integrated n8n workflow. We document five owned functional components and map their handoffs, with the approval retained as an exception.

What would be required before real deployment?

Secure durable storage, access controls, consent/governance and privacy assessment, human review roles, vendor terms, broader evaluation, IRB/ethics review where applicable, and full audit/tracing policy.

What precisely are you requesting today?

Permission to run a synthetic researcher-led pilot, independent scoring help, access to prospective users/buyers, and agreement on KPI measurement before a real-data decision.